



MAJOR *Counseling Theories*

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CNL-500

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Theory A Psychodynamic

OBJECT RELATIONS OVERVIEW

- ★ Object relations: a psychodynamic approach
- ★ View of human nature: we are shaped by our earliest relationships and carry them inside us
- ★ Key concepts: internalized objects, object relations, transference, the unconscious
- ★ Present reactions can be a younger self responding to an old wound
- ★ Why this theory?:
 - It names something I have lived
 - Empirical support: object relations shows direct efficacy evidence (Shedler, 2010)



"One of the many interesting and surprising experiences of the beginner in child analysis is to find in even very young children a capacity for insight which is often far greater than that of adults."

-Melanie Klein

Object relations theory, a psychodynamic approach, holds that personality forms through early relationships with caregivers, which become internalized as mental templates that shape how we relate to others throughout life (Murdock, 2025). This theory sparks my curiosity because it names something I have lived: our earliest relationships become internalized and continue to shape how we relate as adults. These internalized objects operate largely outside conscious awareness and resurface in present relationships through transference. From this view, current distress often reflects earlier relational experiences rather than present circumstances alone. For example, a reaction that feels far bigger than the present situation warrants may be an earlier relational dynamic resurfacing; a present-day trigger can activate a younger internalized self, and as that earlier experience is processed, its grip on the present begins to shift. Shedler (2010) notes that psychodynamic therapy explores aspects of the self not fully known, particularly as they emerge within the therapeutic relationship. Importantly, the goals of psychodynamic work extend beyond symptom relief toward insight, healthier relationships, and a more stable sense of self. Change occurs as these patterns become conscious and are worked through within a safe therapeutic bond.

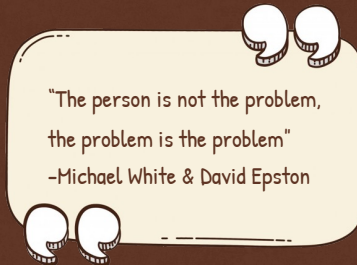
Theory B Narrative Therapy

NARRATIVE OVERVIEW

- ★ View of human nature: people are not their problems; separating the problem from the person creates room to change
- ★ Key concepts: externalizing, problem-saturated story, dominant story/discourse, unique outcomes, re-authoring
- ★ Identity is fluid and can be re-authored
- ★ Empirical Support: Narrative is less researched but growing
- ★ Why this theory?:
 - Externalizing interrupts shame and helps client focus on the problems objectively



Michael White

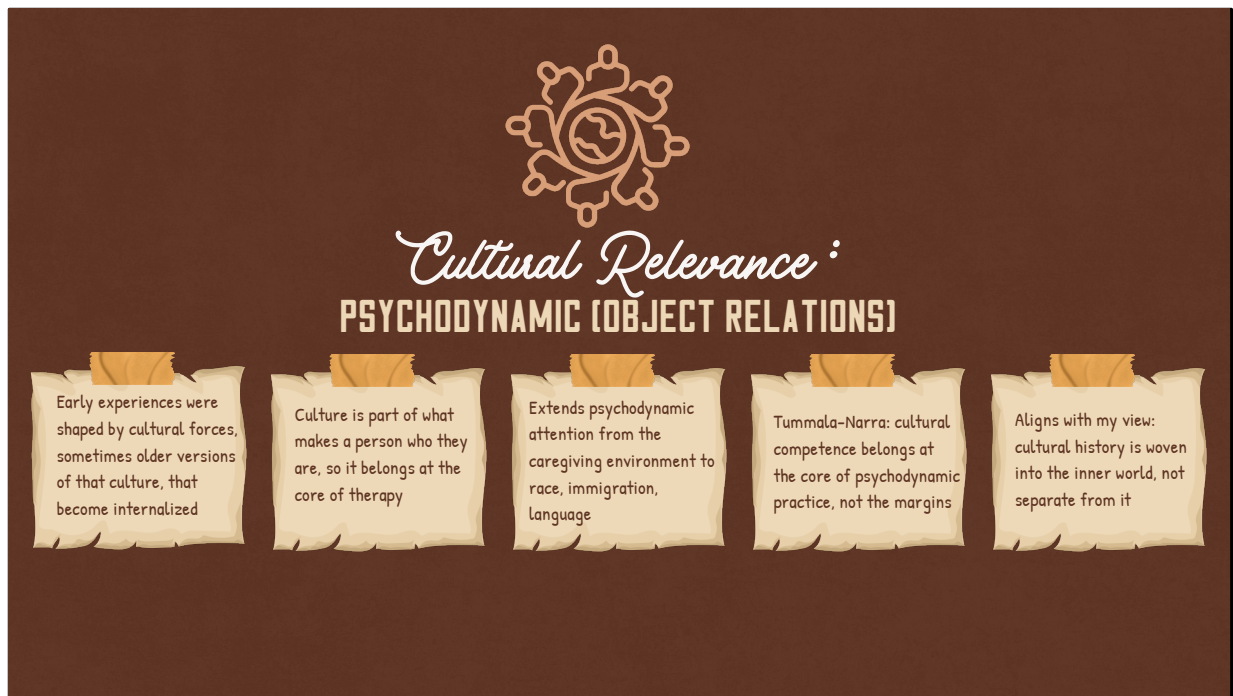


"The person is not the problem,
the problem is the problem"
-Michael White & David Epston

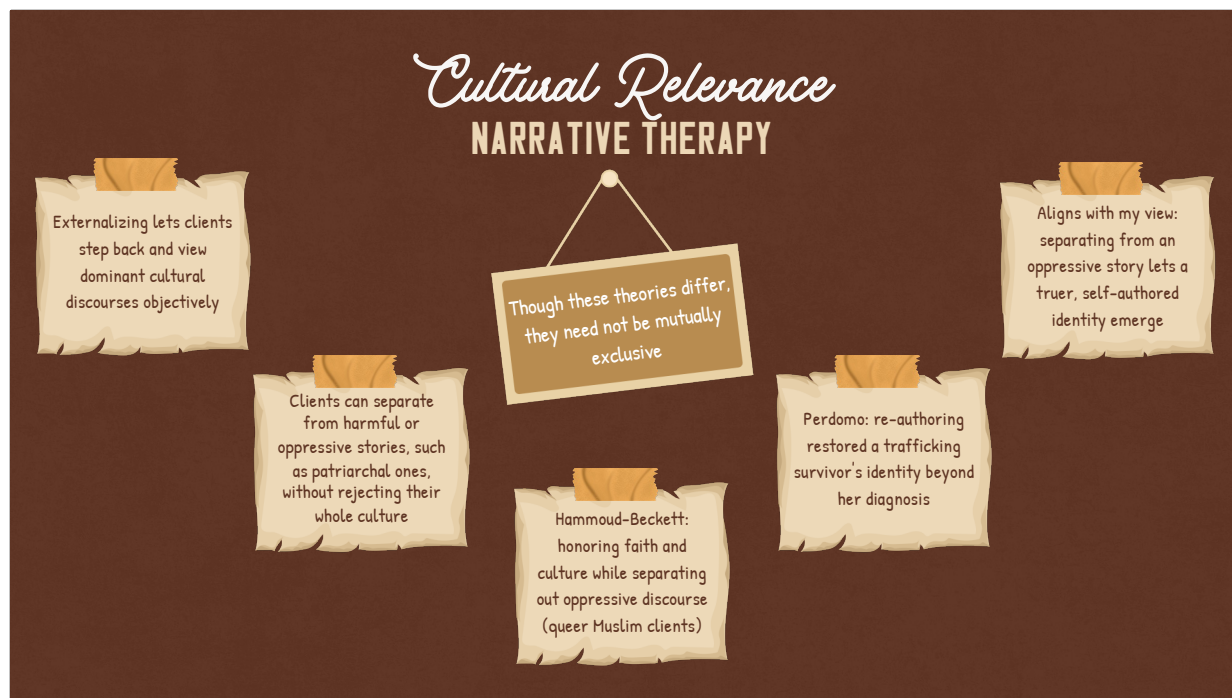


David Epston

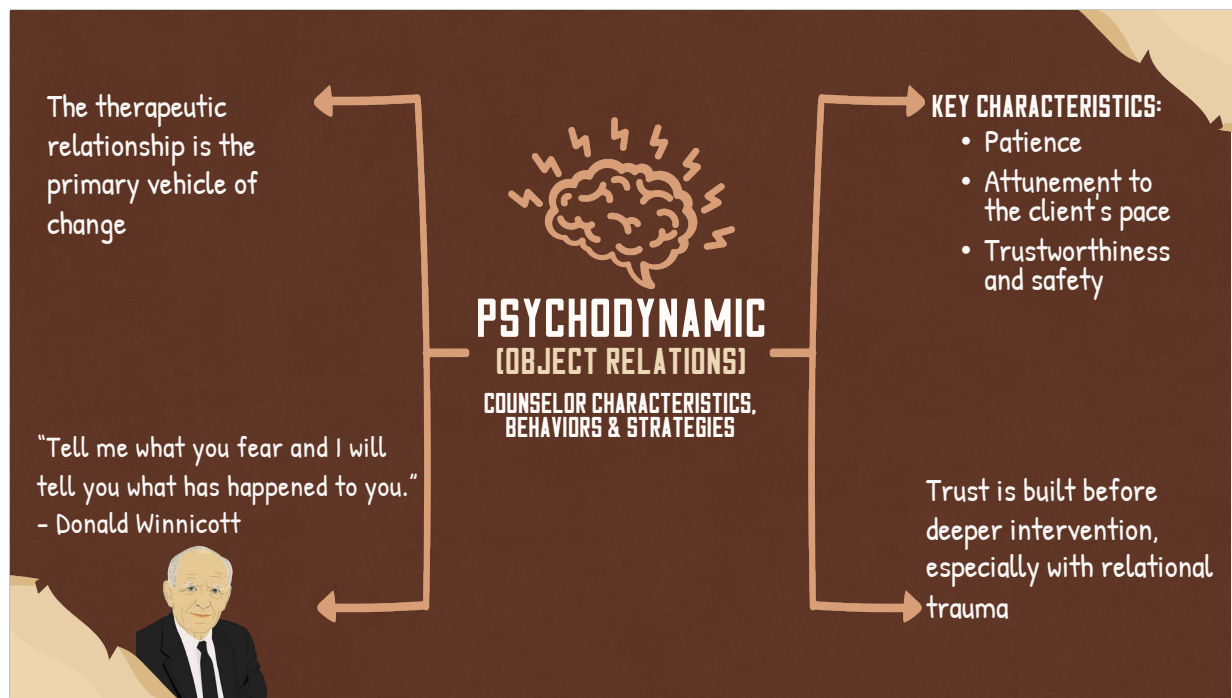
Narrative therapy, developed by White and Epston, holds that people make sense of their lives through stories shaped by family, culture, and dominant social discourses (Murdock, 2025). This framework is clinically compelling because the technique of externalizing systematically interrupts shame. When a problem is separated from a client's identity, they can examine it objectively and extend themselves the same therapeutic grace they would offer to others. When a person's identity becomes fused with a problem-saturated story, they may absorb it as personal failing. A central technique is externalizing, separating the person from the problem, so the problem becomes something to examine rather than an inherent flaw. This shift allows a client to view a situation more objectively rather than through self-blame. The counselor then listens for unique outcomes: moments that contradict the problem story and reveal alternative, preferred identities. Through re-authoring, these exceptions are developed into a new narrative the client prefers. Bruzek (2024) illustrates this in her work with clients labeled with borderline personality disorder, using narrative practices to resist totalizing identity conclusions imposed by a stigmatizing diagnosis and to help clients reclaim identity as fluid and self-authored. Narrative therapy is notably attentive to how dominant cultural discourses shape lived experience, making it well suited to clients whose struggles are tied to marginalization or oppression.



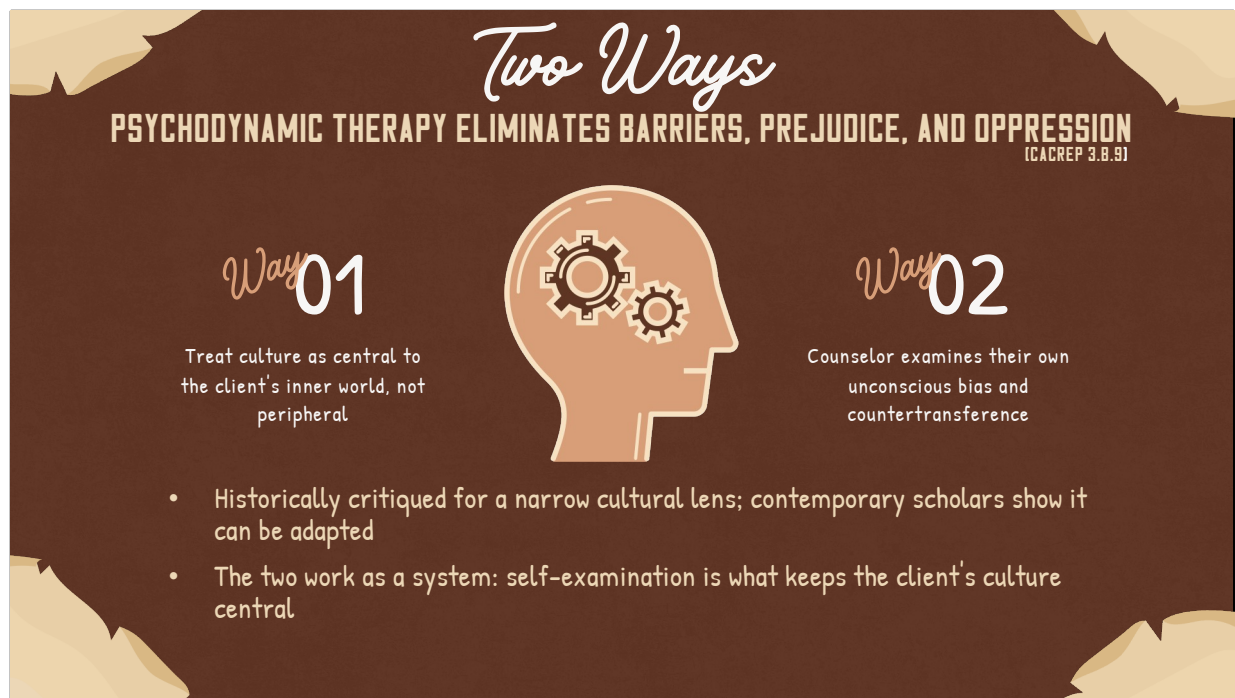
Culture is part of what makes a person who they are, so it should be core to all therapy. In psychodynamic work specifically, earlier experiences were shaped by cultural forces, sometimes older versions of that culture, that become internalized and continue to influence the present unconscious. A psychodynamic approach can surface those imprints. Tummala-Narra (2015) argues that cultural competence belongs at the core of psychodynamic practice rather than at its margins, extending the theory's long-standing attention to the early caregiving environment outward to broader sociocultural contexts such as race, immigration, social class, and language. Because these forces shape the inner world and provide mirroring functions for the self, a theory built to explore internalized early experience is well positioned to explore internalized cultural experience as well. This aligns with my own view of human nature: that we are shaped by our earliest experiences and carry them forward, so a client's cultural history is not separate from their inner world but woven into it. This makes psychodynamic therapy relevant across diverse cultural backgrounds, provided the counselor treats sociocultural context as central to the client's psychic life rather than peripheral to it.



Externalizing lets clients step back and view dominant cultural discourses objectively, separating themselves from harmful or oppressive stories, such as patriarchal ones, without rejecting their whole culture. Hammoud-Beckett (2022) demonstrates this in her intersectional work with queer Muslim clients, honoring their faith and culture while externalizing the oppressive discourses of racism, sexism, and Islamophobia so clients can define their own relationship to them. Jagatdeb et al. (2024) similarly use externalizing conversations in the Indian context to place problems in their social and cultural setting rather than within the individual, resisting internalized blame and shame. Perdomo (2017) shows the same principle with an undocumented immigrant and trafficking survivor, where re-authoring restored the client's preferred values and identity after a diagnostic evaluation had reduced her to her trauma. This also fits my view of human nature: because we all carry earlier versions of ourselves into the present, helping a client separate from an oppressive story lets a more authentic, self-authored identity come forward. Though these theories differ, one locating distress in the past and the other in present narratives, they need not be mutually exclusive; earlier experiences shape the self we bring into each present story. Expanding cultural responsiveness in clinical practice does more than improve therapy; it directly unlocks clients' educational and occupational opportunities. As marginalized individuals move beyond problem-saturated narratives and address internalized systemic trauma, they reconstruct their identities and reclaim the self-efficacy needed to pursue academic milestones and long-term career goals.



In psychodynamic therapy, the therapeutic relationship is not preparation for the work; it is the primary vehicle of change (Shedler, 2010). The counselor prioritizes establishing safety and trust before deeper intervention, recognizing that clients with relational trauma may need considerable time to feel secure. Key characteristics include patience, attunement to the client's pace, and consistent trustworthiness. To facilitate change, the counselor employs the specific strategy of transference interpretation, tracking relational dynamics in real-time. Rather than rushing technique, the effective counselor uses immediate relational processing to point out how past caregiving templates are playing out right now inside the session. This attunement mirrors the theory's core premise: because early relationships shape the internal world, a reparative present relationship can help revise those internalized patterns. Change emerges gradually as the client experiences a safe, reliable relationship, sometimes for the first time, and begins to trust both the counselor and the process. The counselor's steadiness itself becomes therapeutic, offering the client a corrective relational experience that supports lasting change.



While psychodynamic theory has historically been critiqued for developing within a narrow cultural lens, contemporary scholars argue it can be adapted to reduce barriers in two connected ways. First, it treats culture as central to the client's inner world rather than peripheral. Tummala-Narra (2015) argues that cultural competence belongs at the core of psychodynamic practice, extending the theory's attention from the early caregiving environment to broader sociocultural contexts such as race, immigration, and language. This ensures clients from marginalized groups are understood in full rather than misread. Second, psychodynamic work requires counselors to examine their own unconscious assumptions and countertransference. By becoming aware of their own biases, counselors reduce the risk of prejudice entering the therapeutic relationship unseen. These two work as a system: the counselor's self-examination is what makes it possible to keep the client's culture central and to avoid imposing distorted assumptions. Together, they support a more equitable and responsive therapeutic space.

Roles of Counselor and Client **AND CULTURAL RESPONSIVENESS**

- ✓ Client: authority on their own life and active participant in the work
- ✓ Counselor: attuned guide who notices patterns, defenses, and transference the client cannot yet see
- ✓ The counselor gently helps reframe distorted thinking, especially where trauma clouds self-perception
- ✓ The relationship itself serves a corrective function



In psychodynamic therapy, the counselor and client hold distinct but collaborative roles. The client is regarded as the authority on their own life and an active participant in the work, bringing their experiences, history, and willingness to reflect. At the same time, the counselor offers clinical expertise, particularly the capacity to notice patterns, defenses, and transference that the client may not yet see clearly, especially when trauma distorts self-perception. Rather than directing the client, the counselor listens closely and gently helps reframe distorted thinking, asking questions that guide the client toward seeing their situation more accurately. This reflects the theory's premise that insight develops within a trusting relationship. The counselor functions as an attuned guide, and the relationship itself serves a corrective function, allowing the client to internalize a new, healthier relational experience while gradually gaining clarity and authorship over their own story. Cultural responsiveness is maintained by treating the client as the expert on their own cultural experience while the counselor remains reflective about their own assumptions.

Ethical & Legal Issues **ACROSS SERVICE DELIVERY MODALITIES**

(CACREP 3.E.6)

- ACA Code, Section H: distance counseling, technology, and social media (confidentiality, secure platforms, informed consent, competence)
- Online, the counselor cannot control who is off-screen and loses full-body cues, making transference harder to read
- ACA Code, Standard A.4.b: avoid imposing personal values (heightened risk under transference)
- In-person: same duties, but the physical office makes confidentiality, nonverbal cues, and boundaries easier to uphold
- Remote work can also offer comfort and support openness

Ethical and legal issues shift across service delivery modalities. In distance counseling, the ACA Code of Ethics, Section H, requires counselors to protect confidentiality, use secure platforms, obtain informed consent about the format's limitations, and maintain technological competence. These concerns carry particular weight in psychodynamic work, where the therapeutic relationship is the primary vehicle of change. Online, the counselor cannot control who may be present off-screen, and loses full-body cues that inform clinical understanding, making transference harder to read. In traditional in-person counseling, these same duties still apply but are easier to uphold: confidentiality is protected by the physical office, the counselor has full access to nonverbal cues, and boundaries are clearer, which highlights by contrast what distance modalities must work harder to preserve. Equally important is Standard A.4.b, which prohibits imposing personal values on clients. This standard is especially critical in psychodynamic therapy: because transference positions the counselor as a significant relational figure, clients may be primed to absorb the counselor's views as authoritative. Guarding against value imposition protects the client's autonomy and preserves a trusting, client-centered relationship. For some clients, remote work also offers comfort, supporting openness in either modality. Sound ethical practice therefore adapts to the modality while holding these core duties constant.

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